



Modified Essay Questions

SCORING KEY

2012 FELLOWSHIP – AUGUST 2015

INSTRUCTIONS:

- Please use pencil ONLY, preferably 2B
- Do not fold or bend
- Erase mistakes fully
- Make no stray marks
- Completely fill in the oval

Please MARK LIKE THIS ONLY:

0 1 2 3 4

Modified Essay 4

You are working as the junior consultant in a large general hospital. Tony, aged 23 years, was involved in a motorcycle accident in which he sustained multiple internal injuries and an unstable neck fracture. Tony has surgery to stabilise his cervical spine and is currently immobilised in halo traction, has a chest drain, and cannot be moved.

Two days after the surgery, the spinal injury team contacts you expressing concern at Tony's excessive demands for opiates. When you speak to Tony he tells you that he is 'in agony' and can't sleep because of the pain in his back. He looks anxious and low in mood, but denies anxiety or depressive symptoms.

Tony is demanding to smoke cigarettes and cannabis and says that he cannot cope with hospital without them. He has threatened to sign himself out of hospital. He is verbally aggressive towards staff especially when they try to discuss the hospital smoke-free policy with him. The staff would like your help in managing Tony.

Your registrar says the diagnosis is Antisocial Personality Disorder.

Question 4.1

Describe (list and elaborate) the major diagnostic possibilities you would discuss with your registrar. (8 marks)

A.	Personality dysfunction; affect dysregulation eg he has threatened to sign himself out of hospital. He is verbally aggressive towards staff.	0 1 2
B.	Inadequate Pain Management eg he tells you that he is 'in agony' and can't sleep because of the pain in his back.	0 1 2
C.	Substance abuse/dependence disorder eg he is verbally aggressive towards staff especially when they try to discuss the hospital smoke-free policy with him. Demanding to smoke cigarettes and cannabis and says that he cannot cope with hospital without them.	0 1 2
D.	Acute stress disorder eg he looks anxious and low in mood, but denies anxiety or depressive symptoms.	0 1 2
E.	Delirium eg acute severe injury two days post surgery.	0 1 2
F.	No diagnosis (deferred)/interactional difficulties between staff and patient.	0 1 2
G.	Spare (only to be used after approval from Co-Chairs, Writtens Sub-Committee)	1
H.	Did not attempt	0
I.	Writing	0

Note to Examiner: Please mark all bubbles even if the total adds up to more than 8.

Note to Pearsons: Please set the maximum mark to 8.

Marker ID

Initials

ID No.:

1 2 3 4 5 6 7 8 9 10 11 12 13 14 15 16 17 18 19 20 21 22 23 24 25
26 27 28 29 30 31 32 33 34 35 36 37 38 39 40 41 42 43 44 45 46 47 48 49 50



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The information that is presented in italics in this question is a repetition of the earlier sections of the case vignette.

You are working as the junior consultant in a large general hospital. Tony, aged 23 years, was involved in a motorcycle accident in which he sustained multiple internal injuries and an unstable neck fracture. Tony has surgery to stabilise his cervical spine and is currently immobilised in halo traction, has a chest drain, and cannot be moved.

Two days after the surgery, the spinal injury team contacts you expressing concern at Tony's excessive demands for opiates. When you speak to Tony he tells you that he is 'in agony' and can't sleep because of the pain in his back. He looks anxious and low in mood, but denies anxiety or depressive symptoms.

Tony is demanding to smoke cigarettes and cannabis and says that he cannot cope with hospital without them. He has threatened to sign himself out of hospital. He is verbally aggressive towards staff especially when they try to discuss the hospital smoke-free policy with him. The staff would like your help in managing Tony.

Your registrar says the diagnosis is Antisocial Personality Disorder.

Tony gives a history of heavy use of cigarettes (30 per day) and daily cannabis use (2-3 joints per day).

Question 4.2

Discuss (list and debate) what strategies you would advise your registrar might use to help Tony manage without cigarettes and cannabis. (6 marks)

A.	PSYCHO-EDUCATION i. Reasons for staying in hospital, likely length of stay, likely withdrawal, ways he can be helped with these.	0 1 2
B.	BEHAVIOURAL TECHNIQUES i. Relaxation, distraction, positive self-talk, scheduling positive activities (eg visits from friends, video).	0 1 2
C.	PHARMACOTHERAPY i. Ensure adequate replacement of Nicotine patches/gum/inhaler for physiological nicotine withdrawal, Bupropion, Varenicline. ii. SSRI if concomitant anxiety/depression. Short-term benzo-diazepines for acute anxiety. iii. Anti-emetic agents if required.	0 1 2
D.	STAFF MANAGEMENT STRATEGIES i. Positive encouragement from staff. Placing in room with other young adults. Avoiding confrontation while quietly setting boundaries. ii. Education for staff on how to build a collaborative care relationship with the patient.	0 1 2
E.	Spare (only to be used after approval from Co-Chairs, Writtens Sub-Committee)	1
F.	Did not attempt	0
G.	Writing	0

Note to Examiner: Please mark all bubbles even if the total adds up to more than 6.

Note to Pearsons: Please set the maximum mark to 6.

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Question 4.3

Outline (list and justify) the issues you would raise with your registrar in managing Tony's pain. (7 marks)

A.	ASSESSMENT OF PAIN. i. Discussion with surgical team re extent of surgery, post-operative complications, any possible organic contributors to pain. ii. Assessment of pain symptoms. Pain charts, vital signs, medication charts to date.	0 1 2
B.	Psychological factors. Explore and address any psychological contributors to pain such as anxiety, fear, misunderstandings re possible outcomes.	0 1 2
C.	PHARMACOTHERAPY. i. Regular rather than as required analgesia medication. ii. Appropriate pharmacotherapy for co-morbid psychiatric conditions. iii. Do not withhold analgesia on the basis of previous drug misuse.	0 1 2
D.	Nonpharmacological pain management. Occupational therapist input, distraction.	0 1
E.	Staff liaison and education eg pain is genuine and needs adequate relief. Address staff issues eg hostile attitude/countertransference.	0 1
F.	Involvement of Acute Pain Service and/or Drug and Alcohol Service.	0 1
G.	Spare (only to be used after approval from Co-Chairs, Writtens Sub-Committee)	1
H.	Did not attempt	0
I.	Writing	0

Note to Examiner: Please mark all bubbles even if the total adds up to more than 7.

Note to Pearsons: Please set the maximum mark to 7.

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